

CULTURAL HUMILITY IN SUD CARE

Building Trust With Every Patient

La Clínica de Familia | MAT/MOUD & IOP Teams
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Learning Objectives

By the end of this workshop, I will be able to:

PART 1

Explain why cultural competence can reduce patients to checklists

PART 2

Distinguish cultural humility from cultural competence

Identify how my own cultural identities and assumptions enter the clinical encounter

Recognize implicit bias patterns in MAT/MOUD care

PART 3

Connect structural forces to SUD disparities

PART 4

Use Kleinman's explanatory model to elicit a patient's illness narrative

CLOSE

Commit to one concrete practice change I will try this week

How We Work Today

1 Leave the titles at the door.

NP, nurse, admin, IOP staff -- every perspective carries the same weight in this room.

2 Speak from your own experience.

Not from your role, not from "our patients." Use I -- what you have seen, felt, or struggled with.

3 This is a brave space, not a safe one.

Comfort is not the goal. Honesty is.

4 Language has impact, even with good intent.

If a word lands wrong, we correct it with grace -- not judgment.

5 Stay with the discomfort.

We are not here to resolve everything. Not knowing is part of the practice.

6 What is said here stays here.

What is learned leaves.

Before we begin —

Three out of five New Mexicans
know someone with an opioid use disorder.

*Who in this room has been personally affected —
or has a patient whose story has stayed with you?*

PART 1

CULTURAL COMPETENCE

*"I learn your culture
to help you."*

The training model that came before — its promise, and its problem.

What cultural competence training typically teaches

If you understand how this population operates, you can help them change.

Personalismo

Latinos value warm, personal relationships — use small talk to build rapport

Respeto

Patients defer to authority — expect compliance, but be warm about it

Familismo

Family is central — involve family in the treatment plan

Fatalismo

Fatalistic worldview — patient may seem unmotivated or resigned

Marianismo

Women self-sacrifice — may hide their own needs behind family obligations

The problem: these become fixed lenses — predictive rules about 'how Latinos are.' Culture becomes a checklist. The patient becomes a type.

Before we move on — try this.

VIGNETTE

Miguel is 41, Mexican-born, referred to your MAT clinic after an ER visit for an opioid overdose. His wife drove him in. During intake he is polite, makes eye contact, says “sí, doctor” to most questions. He nods when you explain the buprenorphine protocol. He misses his first follow-up appointment.

Discuss with a partner

What's your read on him? What did you actually know versus fill in?

PART 2

CULTURAL HUMILITY

*"I can't be the expert
on your experience.
You are."*

The shift from mastery to practice — and what it asks of you.

Three frameworks — one progression

Cultural Competence	Cultural Humility	Structural Competency
<i>"I learn your culture to help you."</i> Cultural knowledge = predictive rules applied to patients Provider is the expert	<i>"I can't be the expert on your experience. You are."</i> Cultural knowledge = provisional hypotheses to explore with patients Patient is the expert	<i>"What looks cultural is often structural."</i> Cultural knowledge = entry points for structural inquiry System is interrogated, not just patient

Cultural humility: three commitments

Tervalon & Murray-García, 1998

1

Lifelong self-evaluation

Question your assumptions — not once in a training, but throughout your career. The work starts with you.

2

Address power imbalances

Acknowledge the power differential in every clinical relationship — and actively work to reduce it.

3

Community partnerships

Build relationships with communities — not just process patients from them.

Humility is your intention. Safety is theirs to define.

Who do you bring into the room?

You can't practice cultural humility with patients until you've practiced it on yourself.

- ① Fill in your identity wheel — what first comes to mind in each section
- ② Sit with the two reflection questions on the handout
- ③ Share one thing with the person next to you

The seam between you and the system

Implicit bias

Automatic, subconscious associations that influence judgment and behavior — without our awareness, and often contrary to our stated values.

The bias blind spot

Even providers trained to recognize bias consistently rate themselves as more objective than they actually are. Training doesn't eliminate the blind spot — it means you know it exists.

Pronin, Lin & Ross (2002); FitzGerald & Hurst (2017)

Implicit bias lives at the intersection of who you are (Part 2) and the system that trained you (Part 3). Both matter. Neither excuses the other.

Six bias patterns most common in MOUD/MAT settings

Stigma bias	Attribution bias	Therapeutic pessimism
Seeing SUD as moral failure — directly reduces MOUD prescribing (Stone et al., 2021)	“Abuser” in chart = 4× lower odds of OUD treatment plan (Bradford et al., 2024)	15–26% of SUD counselors hold stigmatizing beliefs about their own patients (Avery et al., 2019)
MOUD-specific bias	Racial & ethnic bias	Compassion fatigue
'Trading one drug for another' — bias against medication itself (Dickson-Gomez et al., 2022)	Pro-White, anti-Black/Hispanic/Native bias in treatment decisions and medication access (Lagisetty et al., 2019)	Burnout → depersonalization → patients become cases, not people (Bride et al., 2007)

PART 3

STRUCTURAL COMPETENCY

*"What looks cultural
is often structural."*

The forces upstream of the clinical encounter — and why they show up in your waiting room.

This is our community

Top 3

NM drug overdose death rate

(New Mexico Dept. of Health, IBIS)

3×

Co-occurring SUD + psychiatric disorders: US-born vs. Mexico-born

(Vega et al. 2003)

29%

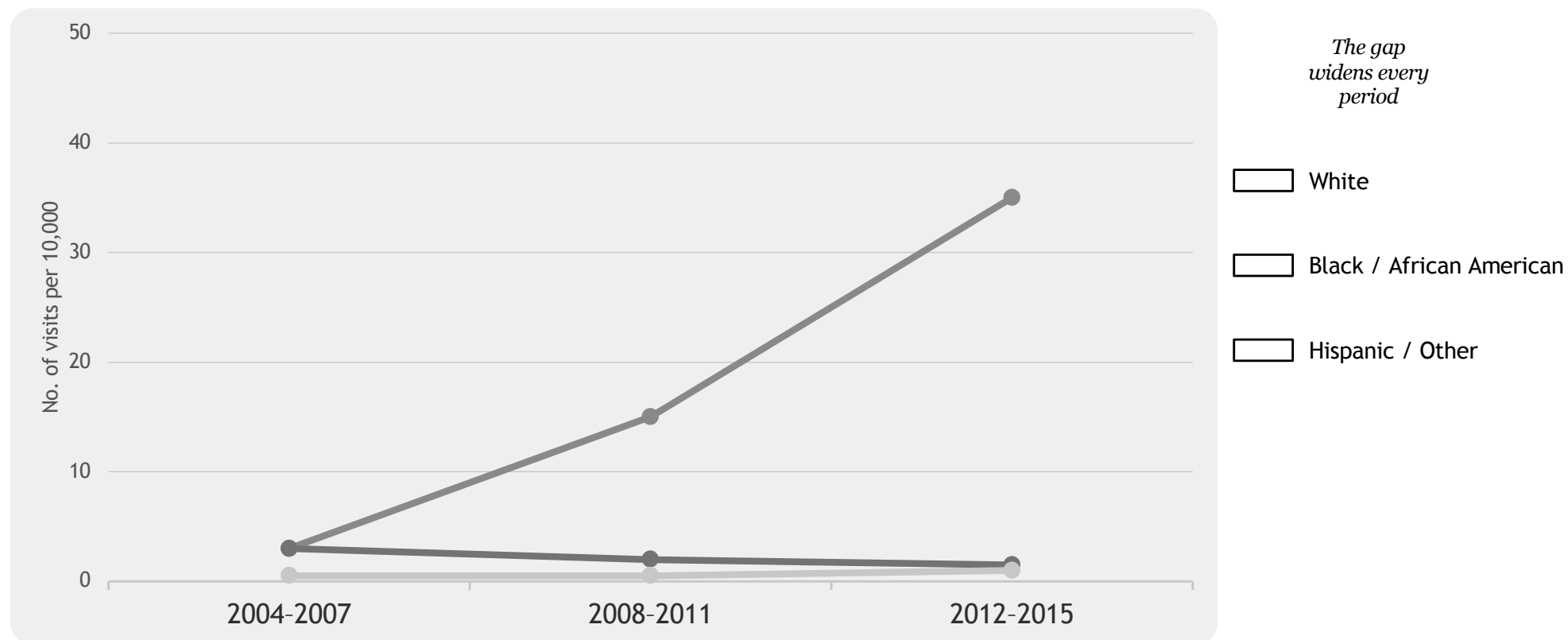
Less likely for Latinx individuals to access specialty SUD treatment

(Hazelden Betty Ford Institute)

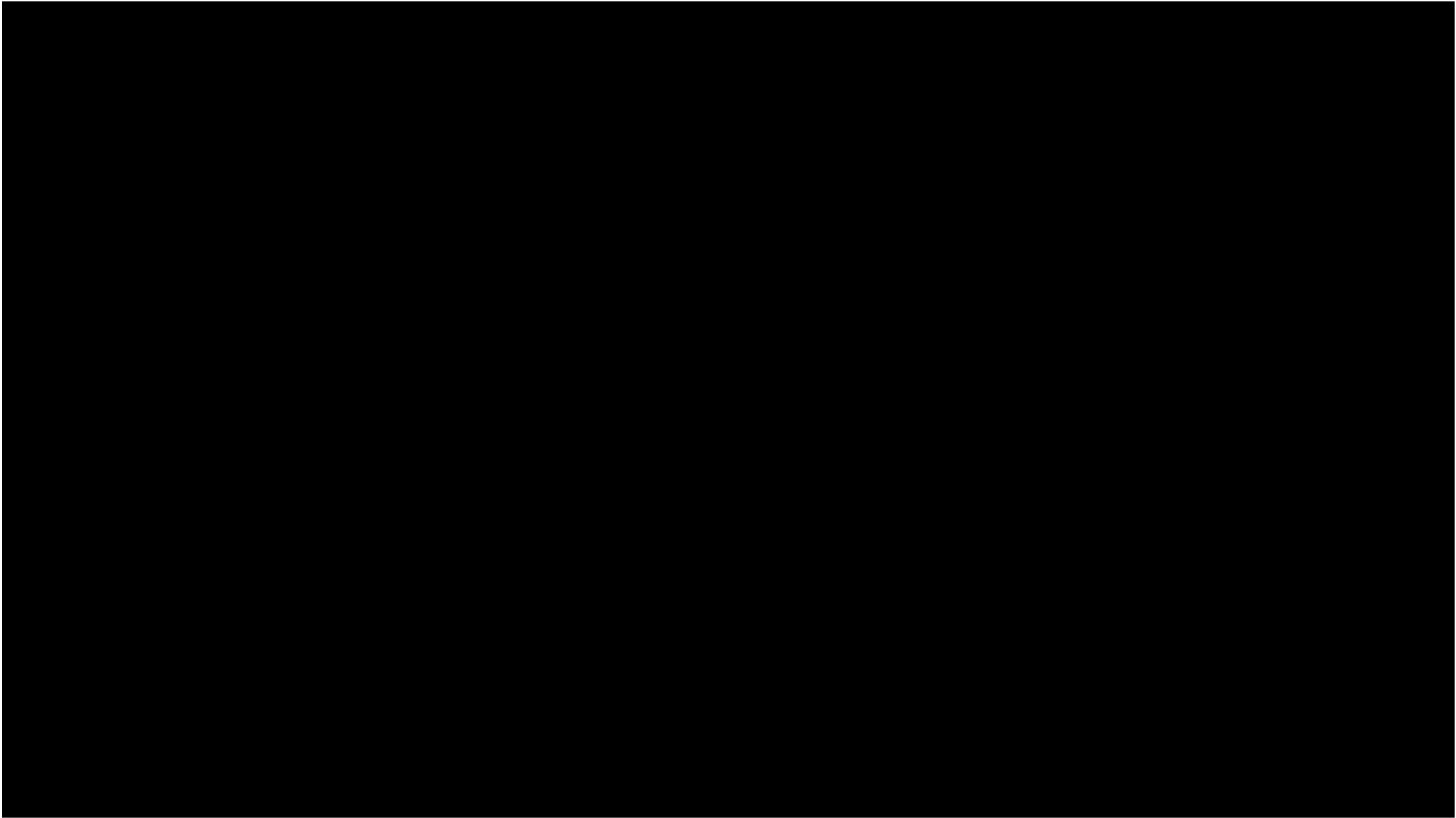
Doña Ana County: 3rd highest OD deaths in NM (2017–2021) | 60,000 undocumented NM residents | NAS more than doubled 2011–2021 | Scott & Andazola — structural competency extended to Doña Ana County health councils and state agencies

Buprenorphine visits by race/ethnicity, 2004-2015

No. of visits per 10,000 visits – nationally representative sample



Buprenorphine visits per 10,000. Source: Lagisetty et al. (2019), JAMA Psychiatry, 76(9), 979-981.



Discussion: look at the lake, not just the fish

Apply the groundwater analogy to your work. Take a moment, then share with the group.

Q1

Think about a patient you've worked with recently — what was in their water?

Q2

Name one thing we do in this clinic — a policy, a form, a requirement — that fixes the fish instead of the lake."

Structural competency: where health actually comes from

UPSTREAM	MIDSTREAM	DOWNSTREAM
• Poverty & income inequality • Immigration enforcement • Historical trauma, ACEs & dispossession • Institutional racism • Lack of safety-net investment	• No transportation to clinic • Language exclusion in services • Daily methadone visits vs. home buprenorphine • Housing instability • Family separation from detention	• 'Non-adherence' • Missed appointments • Apparent fatalism • Low treatment engagement • Premature discharge

What looks like a patient's 'cultural trait' is often a rational response to what's in the upstream and midstream columns.

Metzl & Hansen, 2014

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*“Si Dios quiere,
I’ll get clean.”*

A patient says this to you in your first session. What do you hear?

Three things that phrase might mean

SPIRITUAL COPING

A meaningful framework for agency and resilience. Faith as a resource, not a barrier.

“Si Dios quiere” can carry both acceptance and resistance — not passivity.

EARNED SKEPTICISM

Tried treatment before. Hit walls. This is a realistic read on prior experience, not resignation.

Think of a patient who came in skeptical. What had already happened to them. What did you do with that in the room?

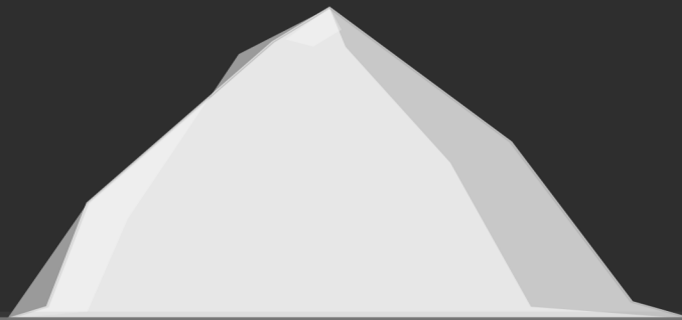
STRUCTURAL REALITY

Immigration status, criminalization, no transportation, fear of the system. Getting clean may genuinely feel structurally out of reach.

What does this clinic make hard for a patient like him — before he even sits down with you?

What's visible in the encounter
is just the surface.

Everything that drives it lives below the waterline.



*Come back to the person you brought into the room at the start.
What's above the waterline in your encounters with them — and
what do you suspect is actually driving it?*

PART 4

THE CLINICAL ENCOUNTER

"So — what is your story?"

Kleinman's explanatory model: how humility, structure, and the patient finally meet in the room.

Kleinman's Explanatory Model: eight questions

Instead of assuming the patient's illness narrative, elicit it.

Culture-specific knowledge becomes vocabulary for asking — not answering.

1	<i>What do you think caused your problem?</i>	5	<i>What do you fear most about your condition?</i>
2	<i>Why do you think it started when it did?</i>	6	<i>What are the chief problems this has caused for you — personally, family, work?</i>
3	<i>What do you think your problem does to your body?</i>	7	<i>What kind of treatment do you think you should receive?</i>
4	<i>How severe is it? Do you think it will last a long time?</i>	8	<i>What results do you hope to get from treatment?</i>

Adding Kleinman to your toolbox

TOOLS YOU ALREADY USE

Motivational Interviewing (MI)

Evokes a patient's own motivation for change through guided conversation.

OARS

Core MI microskills: Open questions, Affirmations, Reflections, Summaries.

FRAMES

A brief intervention framework: Feedback, Responsibility, Advice, Menu, Empathy, Self-efficacy.

All tell you how to communicate.



WHAT KLEINMAN ADDS

Tells you whose worldview you're working within.

It surfaces the patient's *illness narrative* — how they understand their problem, their body, and what help means to them.

Without it, you're working with your assumptions about the patient, not the patient.

The relationship is the treatment

79

Studies confirm therapeutic alliance predicts outcomes across all treatment modalities

(Martin, Garske & Davis, 2000)

$r = .75$

Client-perceived cultural humility and working alliance

(Hook et al., 2013)

Confianza

For Latino patients on pharmacotherapy, therapeutic trust built over time was the primary mechanism driving retention.

(Lewis et al., 2013; Interian et al., 2010)

+30% MAT retention with CHW/promotora support (Massachusetts) | 2× medication adherence with CHW support in NM

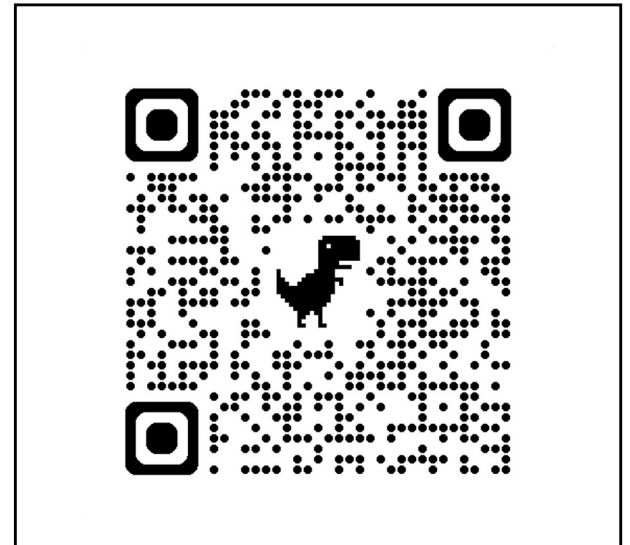
Think of the person you invited into the room at the start. What was different about that relationship — and what did you do, or stop doing, that made it possible?

Your turn.

Groups of 3–4. Mix roles where possible. Select a vignette to discuss.

1 What did your scenario reveal about how the system shapes the encounter, before the provider even makes a choice?

2 What would the patient in your scenario have actually needed from that provider? What would have made it genuinely hard to give them that?"



Holding power with humility

What the provider holds

- The chart and clinical record
- The prescription pad, intake form, or crisis call
- The treatment plan decision
- Discharge authority

The patient knows all of this — and it shapes what they say, and what they don't.

Checking your power

- Invite questions — explicitly
- Name the power dynamic out loud when it helps
- Follow the patient's lead on pace and disclosure
- Make it safe to say what isn't working

Respeto means a patient will agree rather than risk the relationship. Humility means you create space for what's actually true.

Before you leave

Take 3 minutes. Write two things on your card.

1	<i>One thing I am taking away from today.</i>
2	<i>One concrete thing I will try in my next patient encounter.</i>

Lifelong.

Not a training you complete. A practice you return to.

Every patient who stays in treatment — who comes back after a relapse, who tells you the truth even when it is hard — that is evidence that the relationship worked. That someone felt seen enough to stay.

Resources and contact

SAMHSA TIP 59

Improving Cultural Competence in SUD Treatment — free at store.samhsa.gov

NM Dept. of Health

Opioid resources — nmhealth.org/about/phd/idb/opip/

Project ECHO / UNM

Tele-mentoring for rural MOUD providers — hsc.unm.edu/echo

Implicit Association Test

Self-assessment tool — implicit.harvard.edu

Kleinman's Explanatory Model

Pocket reference: The Arthur Kleinman Explanatory Model — <https://www.culturallyconnected.ca/>



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